

Darwin Pathology Protocols and Workflows

There are a number of “sub systems” currently in operation to support the management of patients with chronic diseases or under recall systems. The aim of this document is to highlight these systems and describe the operation of them.

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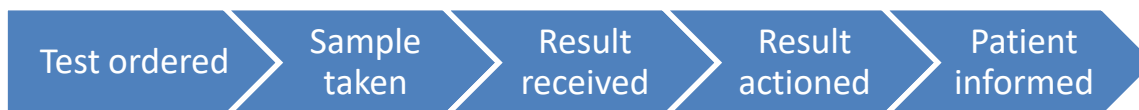
Introduction

Systems to be reviewed:

- Annual review bloods – involving protocols for FBC, U+E, LFT, Lipids and HbA1c
- Smear recall
- PSA monitoring
- Bowel cancer screening
- AAA screening

Annual review bloods:

As part of the Chronic Disease Management System bloods are ordered and then monitored under the supervision of Michele Hassell, Clinical Services Manager.



Results are actioned and patients informed *only* where results need further action or advice.

Patients with normal results are able to obtain those results by utilising the NHS app or ring reception. Clinicians should encourage patients to use the App as the best source of information.

Filtering Workflow > Lab Reports – Daily process in EMIS

Daily process – Lab Reports

Filtering of results

Unmatched

- Check unmatched inbox and match any patients or users. All users to be matched to **Dr Darwin Medical Practice**.

Inbox – Current process up to 4th August 2024

- Select **Workflow View > Global View** and filter the view by **Received** tab
- Change the owner for all the reports that came in the previous day (overnight) and the current day to **Business Admin**. **Only move those that don't have a sticky note, or have not been viewed or with a task present.** **NOTE: Smears are the exception to this, as they may have been moved but not viewed by a nurse, so please check the date and owner / audit history to see if they have already been moved.**
- Another exception to this is the XR (xray) and US (ultrasound) reports**, these should be left with the owner, except if they are on holiday or sick leave. **If the requestor will not be back within 5 working days then distribute using the Clinicians Protocol.** To check the owner is correct, in the report detail, click on the green arrow next to the date under the EMIS report ID section and check the clinician's name in the ordering party. Hover over the person icon to the right of the report and check the name of the clinician owning the report, matches that from the report ordering party.

Inbox - Process that will be in place from 5th August

- Select **Workflow View > Global View** and filter the view by **Received** tab
- Change the owner for all the reports that came in the previous day (overnight) and the current day to **Business Admin**. **Only move those results that have not been viewed. All results that the Business Admin team have processed and already sent to a clinician will show as Viewed.**
- Sticky Notes will no longer be used as an indication for moving labs.**

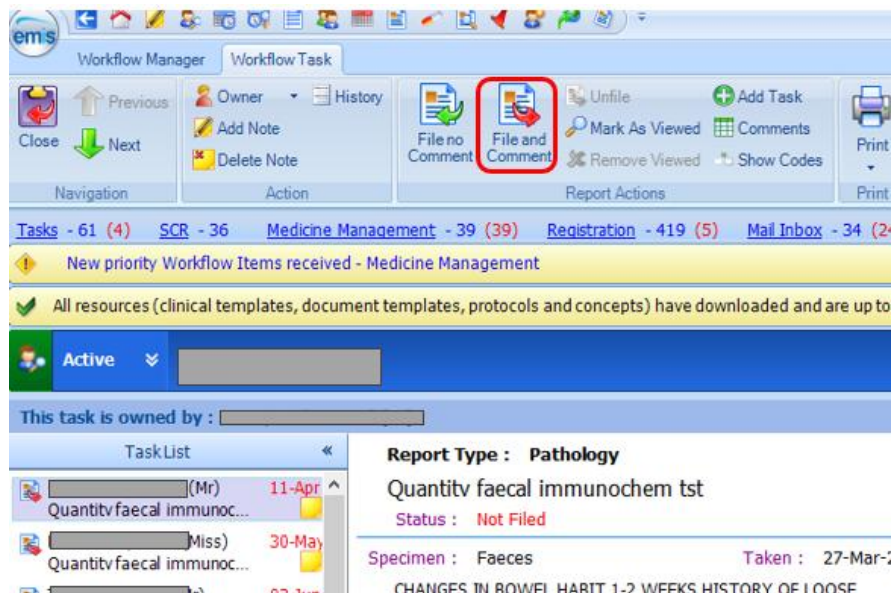
Received	Patient	Report	Viewed	Filed	Tasks
24-Jul-2024		Urine microscopy			
24-Jul-2024		Urine microscopy			
24-Jul-2024		Urea and electrolytes			
24-Jul-2024		Urine microscopy			
24-Jul-2024		Urea and electrolytes			
24-Jul-2024		Urea and electrolytes			
24-Jul-2024		Full blood count - FBC			
24-Jul-2024		Microscopy			
24-Jul-2024		Haemoglobin A1c level			
24-Jul-2024		Full blood count - FBC			
24-Jul-2024		Urine microscopy			
24-Jul-2024		Urine microscopy			
24-Jul-2024		Serum testosterone			
24-Jul-2024		Microbiology			
24-Jul-2024		XR Foot Rt			

- An exception to this is the XR (xray) and US (ultrasound) reports**, these should be left with the owner, except if they are on holiday or sick leave. **If the requestor will not be back within 5 working days then distribute using the Clinicians Protocol.** To check the owner is correct, in the report detail, click on the green arrow next to the date under the EMIS report ID section and check the clinician's name in the ordering party. Hover over the person icon to the right of the report and check the name of the clinician owning the report, matches that from the report ordering party.

Processing of results – To be introduced from 5th August

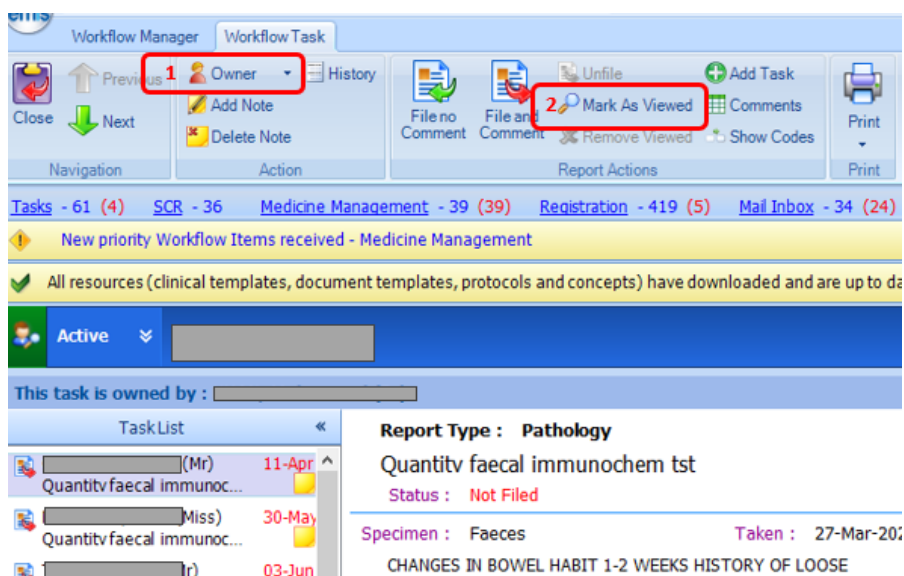
This process will only be in place from 5th August

- The Business Administrators will work through the labs in their inbox following this protocol to process the lab reports on a daily basis.
- When processing the results, where the result is normal or within range as per the protocol, the team will use the **File and Comment** option;



- Where results need to be forwarded to a clinician, the team must **first change the owner using the Owner button and then secondly click on Mark As Viewed**

NOTE: If Mark As Viewed is selected first, the system will drop down to the next patient in the viewing panel on the left hand side. You will need to click back on to the correct patient to then change owner.



- Sticky notes are not required for all lab results. Only add a sticky note if there is a query on the report you are forwarding to the clinician that is not part of the protocol for that lab result.

Weekly Check of Global View – Monday task

On a weekly basis the Business Administrators will carry out a review of the full Global Lab Reports View. This check will be carried out on a Monday.

- Select **Workflow View > Global View** and filter the view by **Received tab**
- This task is to locate any labs older than 1 week that have not been actioned.
- Specifically looking for;
 - Results sat with a particular clinician that have not been processed for a number of weeks / months – Raise this to the **Clinical Services Manager**

	21-Jun-2024		Quantitv faecal immunochem tst	✓	✓		
	06-Jun-2024		Quantitv faecal immunochem tst				
	03-Jun-2024		Quantitv faecal immunochem tst				
	30-May-2024		Quantitv faecal immunochem tst				
	11-Apr-2024		Quantitv faecal immunochem tst				

- Results sent to a clinician that is now off work for a longer period of time – these results will need to be moved to another clinician for processing – it may be helpful to add yourself as a deputy for any known clinicians that will be off to ensure all labs are moved / actioned for that clinician.

Tips

- Any results that have been viewed, filed and have a task attached but have an old date are OK to leave as these are still in process by the clinician

	21-Jun-2024	(Mr)	Quantitv faecal immunochem tst				
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- Check **Sticky notes** for any old results that have not been viewed or filed, as the note may advise that the clinician is awaiting guidance, or another set of results before they process it. In the below example this result has not been viewed or filed but the sticky note advises the clinician is awaiting advice and guidance response. These are OK to leave.

22-Jun-2024 Urea and electrolytes

await a/g response

🔪NOTE: Where you locate more than one old unactioned result owned by a particular clinician it may be useful to add yourself as a deputy to that clinician and view their labs inbox in isolation to get a full view of how many are pending with that clinician and action.

NOTE: From 5th August all results will be shown as viewed as the Business Administrators will be using this method as part of the processing of the results.

Clinicians Protocol – guidance for forwarding of results

Clinicians Protocol

1. On Consultations check which clinician requested the test and send to the requestor (This can be Trainees (Not FY Trainees) & Urgent Care Team but not Locums)
2. If requestor on leave, Send to Usual GP
3. If Usual GP is on leave distribute between those GPs in work

Check time that the report is being sent – if early AM send to AM clinician, late morning send to PM Clinician / PM send to tomorrow's AM clinician

Clinical Services Manager – when on a rest day or on holiday, please distribute to another clinician using this protocol.

 **NOTE:** If there are any tests that are not on the usual list, always send to the requestor. Unless the requestor is on leave.

Care Home Protocol – guidance for repeating of bloods for care home patients

Care Home Protocol

Nursing Homes (Grangemoor / The Lodge / Seaton Court / Chase View)

Responsible for taking the bloods of residents themselves.

- Call the home and ask that they repeat the test for the patient

Residential Homes (Bracken House / Hammerwich Hall / Watford House)

Check who took the blood results – District Nurse or Home Visit via the practice?

- If District Nurse – send to **Clinical Services Manager** to send a referral for the repeat test
- If home visit via the practice (usually KM or PK) Book a slot for the patient on the next available Friday Home Visit appointment slot (usually under KM or PK) Add a comment to the slot to advise retest required

Follow Up Appointment Protocol – guidance for self book links

Follow Up Appointment Protocol

Where possible if follow up is required send the patient an Accurx message with self-book link.

1. Locate the patient in EMIS and then from **Accurx Toolbar** select **Message**
2. Next choose a Template message and then select a **Booking Link** to add a self-book link - Use the table below for examples of slot types & templates for associated examples;

Reason for retest	Slot type to be selected in Booking Link	Template for message text
U&Es - eGFRcreat	For annual Review 'Chronic Disease Annual Review with nurse'	Chronic Disease Annual review Invitation
	For repeat blood test 'Blood test, weight, height, BP'	GFR Repeat
Lipids – Cholesterol	- 'Medication Review F2F with PCN' or 'Medication Review Telephone with PCN'	Medication Review following blood test
Hba1C 42.5 – 48	'Health Check' with Nurse Associate	Hba1c elevated glucose
Hba1C >48-59	For repeat Hba1c test use 'Blood test, weight, height, BP'	Raised Hba1c repeat
Hba1C above 59	For review with Pharmacist 'Medication Review F2F with PCN' or 'Medication Review	Medication Review following blood test
Thyroid monitoring - TSH	To rebook to have Thyroid monitoring repeated within 6-8 weeks	Repeat TSH

 **NOTE:** If there are no appointments available for the selected slot type or if the patient does not have a mobile number, a Task will need to be sent to Reception to ask them to book instead. Result will then need to be filed as 'Task Sent'

Booking Note for Dr Slots Protocol – guidance for booking note for dr slots

Booking Note for Dr Slots Protocol

zzz On-call GP AM Greenwood (08:30-12:40)

RM9

	Time	Description / Patient Name
▲	08:30	sick note request
▲	08:35	sick note request
▲	08:40	sick note request
▲	08:45	sick note request
▲	08:50	sick note request
▲	08:55	sick note request
👤	09:00	doctor available for supervision and queries
👤	09:10	doctor available for supervision and queries
👤	09:20	doctor available for supervision and queries
✍️	09:30	note for dr
✍️	09:40	note for dr
✍️	09:50	note for dr

Note for Dr slots are available with the AM and PM on-call Doctor each day.

1. The slots should be booked in time order.
2. When all the am slots are booked moved to the pm. They can be booked in the morning for the afternoon slots. Do not book into slots after that appointment time has passed.
3. If all the note for doctor slots have been booked that day it is still imperative that urgent bloods are actioned. Please book in as an extra slot at the end of the day. In the reason state "urgent blood result for today". If it is after 5pm then please also contact the on call doctor, either by use of urgent screen message, or in person/phone to ensure they see the need to act that day.
4. No extra slots should be added to the morning session.
5. Do not change any other slot to note for doctor without agreement from the doctor concerned.

MyBot GP has been implemented to start automatically processing normal lab results.

The current list of tests that the Bot is processing are below.

The Business Admin team will still process the abnormal results of these tests.

List up to date as at 22.01.26

Hba1C

Thyroid

Glucose

BNP

Ferretin

ESR

B12 and folate

FBC

Additional checks for labs whilst MyBotGP is being integrated

- Labs will be moved as they are currently from the Global inbox to Dr Darwin Medical Practice, skipping the normal results for the tests listed above
- The team will process the labs as normal from the Dr Darwin Inbox
- After completing the labs, the team will view the Global in box again for any missed labs in the initial move and check for any normal labs that the Bot should have processed, but it has left for 24 hours. The team will file these and not wait for the Bot

NOTE if any results appear with BDH and show as Download to Copy these have been requested via ICE. Check consults / audit history to see who requested and forward to that clinician.

Check Hb

- Check Hb level, if normal (Men 130-170 Women 120-150) proceed to check MCV
- **If abnormal; above or below the normal ranges ;** forward to clinician as per **Clinicians Protocol**

Check MCV

- Check Mean corpuscular volume (MCV), if normal proceed to check WCC
- **If abnormal; above or below the normal ranges ;** forward to clinician as per **Clinicians Protocol**

Check Total WCC

- Check total WCC level, if normal (4.0 - 10.0) check platelets
- If abnormal add sticky note and forward as per **Clinicians Protocol**.

Check Platelets

- Check Platelets, if normal (**150-475**) check Neutrophils.
- If **below 150** check to see if abnormal previously, if known and results stable (range +/- 20) then check Neutrophils. If further change of **more than 20 lower** then, add sticky note and forward as per **Clinicians Protocol**. If further change is higher than previous, check Neutrophils.
- If **below 150** and the previous result was normal forward as per **Clinicians Protocol**
- If platelets **over 475** forward as per **Clinicians Protocol**.

Check Neutrophils

- Check Neutrophils, If normal check if ESR, if no ESR file.
- If **count below 2** forward as per **Clinicians Protocol**
- If **count higher than upper limit** forward as per **Clinicians Protocol**

Check ESR

- **If 10 above upper limit** check previous level if above previous level by more than 10 add sticky note and forward as per **Clinicians Protocol**. (RA patients being monitored may have persistent elevated level) Where the figure is lower than previous, file.
- If normal file.
- If there is **no reading** due to the test not being completed or due to technical difficulties forward to the requestor **and add sticky note** asking if the test should be repeated.
 - If the clinician replies to do the test again, sent a task to reception specifically asking for the **ESR test** to be repeated.

Need to check each section, even if an action is found on the first item. Make any notes as you process each one, to know what to add as results are filed. **Keep an eye out for the AKI – they don't occur very often but need to be actioned urgently if they appear.**

NOTE if any results appear with BDH and show as Download to Copy these have been requested via ICE. Check consults / audit history to see who requested and forward to that clinician.

Sodium (Na)

- Review Na level, if normal move on
- If between **128 - 133**, and 1st test, the test will need to be repeated within 7 days ;
- Check the **Appointment book** to see if they have an appointment booked with Nurse. If booked file and add **"Appointment already booked - follow up as planned - added note"**. Locate the appointment and **add a note to repeat U+Es**.
- If no appointment booked type **"Task sent"** in comments *[Select do not display on patient records]* and send **Urgent** task to reception with request to "Please book for repeat U+Es within 7 days. Nothing to worry about for patient"
- If 2nd result **128 - 133**, add sticky note forward using **Clinicians Protocol**
- If **lower than 128** book a **Note for Dr slot using the 'Note for Dr protocol'** for the on call doctor for action that day
- If **higher than 146** book a **Note for Dr slot using the 'Note for Dr protocol'** for the on call doctor for action that day

Potassium (K)

- Review K level, if normal move on
- If **3.0 - 3.5** or **5.5 - 5.8**, and 1st test repeat within 7 days;
- Check the **Appointment book** to see if they have an appointment booked with GP or Nurse. If booked file and add **"Appointment already booked - follow up as planned - added note"**. Locate the appointment and **add a note to repeat U+Es**.
- If no appointment booked type **"Task sent"** in comments *[Select do not display on patient records]* and send **Urgent** task to reception with request to "Please book for repeat U+Es within 7 days. Nothing to worry about for patient"
- If 2nd result the same, add sticky note, forward using **Clinicians Protocol**
- If **below 3.0 or above 5.8**, book a **Note for Dr slot using the 'Note for Dr protocol'** for the on call doctor for **action that day** - add a sticky note and forward to the clinician

Chloride (Cl)

- If **above 90** move on
- If **below 90** add sticky note and forward using **Clinicians Protocol**

eGFRcreat (GFR)

- If **above 60** file result, **"within acceptable limits"**
- If **between 30 - 60**, check if **CKD** coded and known, if stable then add **"GFR stable in known CKD"** and file
 - If GFR has dropped **more than 10 points**, forward using **Clinicians protocol**
- If **between 30 - 60**, **no CKD** documented and **1st low** reading then check **Appointment Book** to see if patient has appointment booked.
 - If appointment booked file and add **"Appointment already booked - follow up as planned"**.
 - If no appointment booked send an Accurx self-book to the patient to book for routine repeat blood test **See Appointment Follow Up Protocol**. File as Accurx sent
 - If **between 30 - 60**, no CKD documented and is 2nd test then send to **Clinical Services Manager**
 - If **below 30** no CKD4/5 coded send to **Usual GP (use Clinicians protocol point 2 onwards)**

AKI

- If **any reading** send to Dr urgently, book a **Note for Dr slot using the 'Note for Dr protocol'** for the on call doctor for **action that day**

Se N-term pro BNP

- **Any reading above 400** forward using the **Clinicians Protocol**

NOTE if a patient has already had two repeat thyroid bloods and another is due please forward using the **Clinicians Protocol**

Check TSH

- If TSH normal, file
- If TSH abnormal ;
 - if **below the lower range** indicator forward using the **Clinicians Protocol**
 - if **between 4.2 to 10** repeat in 6-8 weeks - send the patient an Accurx, see **Appointment Follow Up Protocol**. File as Accurx sent. *Where this is a repeat test result and still falls in this range, send an Accurx for the patient to book in for a repeat in 6-8 weeks.*
 - If **above 10** forward using the **Clinicians Protocol**

Check T4

- If T4 normal, file
- If TSH below 10, and no T4 check **Appointment Book** to see if they have an appointment booked for annual review for T4 to be repeated.
 - If appointment booked, file and add "**Appointment already booked - follow up as planned**".
 - If no appointment booked then T4 needs to be repeated in 4-8 weeks. Send the patient an Accurx Self-book link for a F2F Blood Test **See Appointment Follow Up Protocol**. File as Accurx sent
- If T4 abnormal send the patient an Accurx Self-book link either Phone Call or F2F for a review with a **Pharmacist** **See Appointment Follow Up Protocol**. File as Accurx sent

Protocol for B12, Folate, Ferritin

Last Updated: 26.2.24

Check B12

- If **normal** or **high (up to 1000)** move to folate
- If **above 1000** forward using the **Clinicians Protocol**
- if **low (below lowest range)** forward using the **Clinicians Protocol**

Check Folate

- if normal or high (No range set) file
- If **low** forward using the **Clinicians Protocol**

Check Ferritin

- If normal, file
- If either **high (50 points above higher range)** or **low (anything below lower range)** [NOTE: the ranges are different for Males & Females] forward using the **Clinicians Protocol**

Se Prostate specific Ag level (PSA)

Last Updated: 7.3.24

Se Prostate Level

- If normal, file
- **Anything outside of normal range** (different for each lab) forward using the **Clinicians Protocol**

Vitamin D Protocol

All results must be checked for this type of report. Some will show as normal, but in fact are abnormal. Always check the number.

Vit D above 50

- Result normal, no further action, file

Below 50

- Forward using the **Clinicians Protocol**

Bilirubin

- If normal move to next set of markers
- If **between 21 and 28** then review and see if **Gilberts Syndrome** (check Problems). If not add sticky note and forward using the **Clinicians Protocol**. If it is Gilberts move to next marker
- If **above 28** then add sticky note and forward using the **Clinicians Protocol**

Serum Alk Phos ALT

- Review this set of markers, if normal move to next set of markers
- If abnormal and **more than three times upper limit of normal** then add sticky note and forward using the **Clinicians Protocol**
- If abnormal and **lower than the lower limit of normal** forward using the **Clinicians Protocol**.

Albumin Globulin

- Review this set of markers, if normal move to next set of markers
- If abnormal then add sticky note and forward using the **Clinicians Protocol**

Total Protein

- If normal move to next set of markers
- If abnormal and **below 55** then add sticky note and forward using the **Clinicians Protocol**

Adjusted / Corrected Calcium

- **Less than 1.8 - potential emergency**, book a **Note for Dr slot using the 'Note for Dr protocol'** for the on call doctor for action
- **1.8 - 2.14** - mild hypocalcaemia, organise repeat Calcium within 1/52, file. If second test remains low, forward using the **Clinicians Protocol**
- **2.15-2.55** - Normal range, move to next marker
- **2.56 - 2.7** - mild hyper calcaemia, repeat in 1-2/52. If second test high, add sticky note and forward using the **Clinicians Protocol**
- **Above 2.7** - add sticky note and forward using the **Clinicians Protocol**
- **Above 3.0** - book a **Note for Dr slot using the 'Note for Dr protocol'** for the on call doctor for action

Gamma GT

- If normal file results
- If abnormal and **more than three times upper limit of normal** then add sticky note and forward using the **Clinicians Protocol**
- If abnormal and **less than three times** normal file results

Serum inorganic phosphate

Last Updated: 9.6.26

Serum
inorganic
phosphate

- If normal file results
- If **abnormal** then add sticky note and forward using the **Clinicians Protocol**

Serum Urate Level

Serum
Urate

- If normal file results
- If **abnormal** then add sticky note and forward using the **Clinicians Protocol**

UALB / CRE Ratio or U Protein/Cre Ratio

UALB /
CRE

- If normal file results
- If **abnormal** then add sticky note and forward to the **Clinical Services Manager**

Se CA 125 Level (this will be for Females only)

Se CA
125

- If normal file results
- If **abnormal** then add sticky note and forward to the **Requestor. If the requestor is off send to On Call Dr / Usual GP as this needs to be actioned same day.**

Serum C reactive protein level (CRP)

Serum C reactive
protein level

- If **75 or over** (regardless of RA) send to **On Call Dr**
- If **above 10 (and below 75)** confirm previous readings. If stable against last reading and has RA file.
- If **no RA and above 10** forward using the **Clinicians Protocol**

Serum Triglycerides

- Whilst this protocol is not in order as is on screen, scan down and check the Serum Triglycerides first as if they are abnormal it will need to be sent to a clinician
- if **3.5 or above** forward using **Clinicians Protocol**

Serum cholesterol

- Review total cholesterol, if **less than 5** file with "**within acceptable limits**" code
- If **above 5** then check to see if patient on register for **CHD, Stroke, Diabetes or Cardiovascular**, if they are check Medication to see if on Statins [**TIP: if 'Statins recommended' appears in pink pop up then patient is not on statins**]. If patient is on register and on Statins send to **Clinical Services Manager**
- If patient is not on register but is on Statins file and add "**Chol (Reading) on Statin**"
- **NOTE: At this stage check appointments / consultations / text search on care history to see if anything documented for the patient to see if they have an appointment / have refused statins etc.**
- If have an appointment file as "**Appointment already booked - follow up as planned**".
- If **no** appointment booked, send the patient an Accurx to self-book link to book review with **Pharmacist See Appointment Follow Up Protocol**. File as Accurx sent
- If patient has declined Statins file and add "**Chol (Reading) Statin declined. Please discuss at next review**"
- If **above 5** but **not on** register or statins, then proceed to QRisk section

Calculate QRisk score

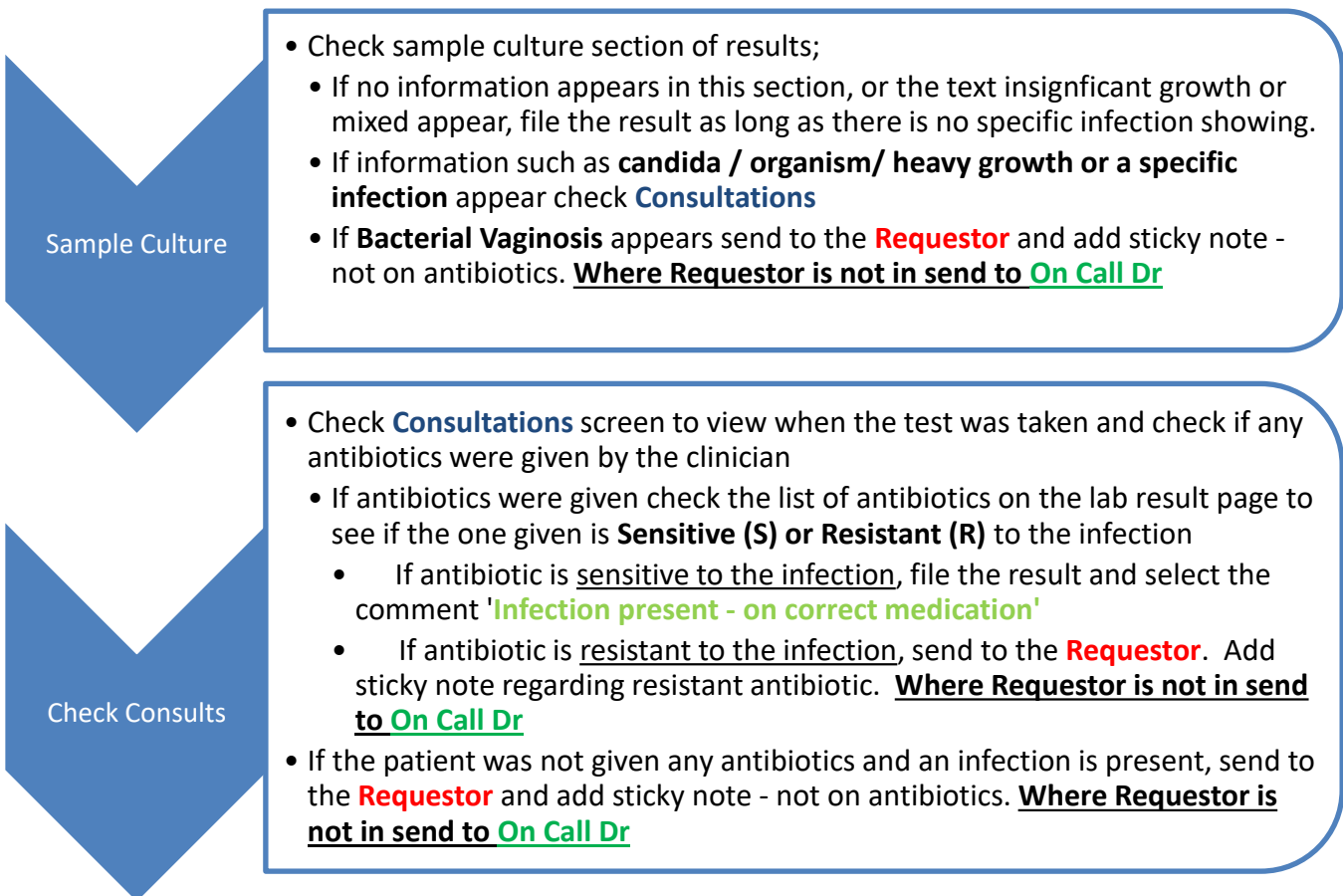
- **QRISK cannot be used for age 24 and under or 80 years +**
- Calculate Qrisk > **Care History** > **Add** > **Data Using Template** > search for **Qrisk Calculation** and double click > Change to **Other Note** > If doing a lot can tick *Store for Session* > Within the QRisk window click on **Calculate** next to **Qrisk CVD** risk at the top > make a note of the % score
- If QRisk **above 10%** move to prescribing review section
- If QRisk **below 10%** add "**within acceptable limits**" code and file
- If unable to calculate Qrisk move to prescribing review

Prescribing review

- Whilst in **Care History** complete a text search for **Statins** as patient may have declined or not tolerated previously
- Check **Medication** to see if on **Statins**
- Check the **Appointment book** to see if they have an appointment booked has an appointment booked;
 - If have an appointment file as "**Appointment already booked - follow up as planned**".
 - If **no** appointment booked, send the patient an Accurx self-book link to book review with **Pharmacist. See Appointment Follow Up Protocol**. File as Accurx sent.
- If patient has declined Statins file and add "**Chol (Reading) Statin declined. Please discuss at next review**"
- If in any doubt send to **Clinical Services Manager**

All results must be checked for this type of report. Some will show as normal, but in fact will need treatment.

NOTE: Swabs / microbiology results should go back to the requestor – this includes the Urgent Care Team.



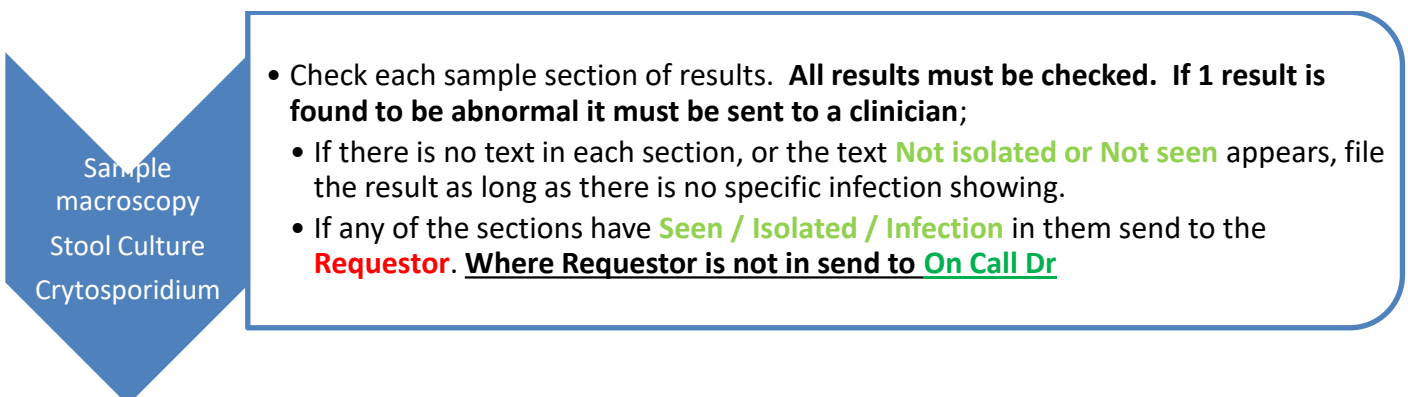
Macroscopy, Stool culture, Cryptosporidium microscopy

Last Updated: 12.11.24

All results must be checked for this type of report. Read all text within each of the results.

Where positive results are found by the lab, a note to say that it has been sent to Swansea may appear and then a second updated result will be sent for the patient.

NOTE: Results should go back to the requestor – this includes the Urgent Care Team.



42.5-47
(6.1%-6.4%)

- Review coding - if pre Diabetic (Non-Diabetic Hyperglycaemia) or Diabetic coded and pt aware, file result and add **either** "Known Pre Diabetes - Stable" for Pre Diabetic or "Stable Diabetes control- well done" for Diabetic
- If not coded then check **Appointment Book** to see if patient has appointment booked with nurse for AR. If appointment booked file and add "Appointment already booked - follow up as planned". If no appointment booked send the patient an Accurx self-book link to book Health Check with Nurse Associate. **See Appointment Follow Up Protocol**. File as Accurx sent.

>48-59
(>6.4 % - 7.5%)

- If pt known diabetic then add "Stable Diabetes control- well done" to result, file.
- If pt is not a known diabetic check **Appointment Book** to see if they have an appointment booked with GP or Nurse.
 - If appointment booked, file and add "Appointment already booked - follow up as planned".
 - If no appointment booked then hba1c need to repeated in 4-8 weeks. Send the patient an Accurx Self-book link for a F2F Blood Test **See Appointment Follow Up Protocol**. File as Accurx sent
- If no diagnosis and this is the second reading after repeat bloods, and also in this range, check the **Appointment book** to see if they have an appointment booked with GP or Nurse. If booked file and add "Appointment already booked - follow up as planned". If no appointment booked type "Task sent" in comments *[Select do not display on patient records]* and send **Urgent** task to reception with request to "book review with GP" for diagnosis discussion

59 - 99
(above 7.5%)

- If above 59 and known diabetic, check the **Appointment Book** to see if there is a follow up booked. If appointment booked file as "Appointment already booked - follow up as planned"
- If no appointment booked, check if under DM nurse or secondary care follow up (check Docman ShareView for any letters from *Diabetic Clinic not Retinopathy*), if so file as "Under Hospital follow up an monitoring, no action"
- If not under secondary care then file and send the patient an Accurx Self-book link for review with Pharmacist. **See Appointment Follow Up Protocol**. File as Accurx sent
- If not a known diabetic, repeat Hb1ac if first time high, file and type "Task sent" in comments *[Select do not display on patient records]* and send **Urgent** task to reception with request to book repeat blood test **within 2 weeks**
- If no diagnosis and this is the second reading above 59 then file and type "Task sent" in comments *[Select do not display on patient records]* and send **Urgent** task to reception to book with GP for discussion on diagnosis noting that this can't be a routine appointment as more urgent the patient is seen.

Over 100

- If **Over 100** forward using the **Clinicians Protocol**

Normal result

- If normal result check [Consultations](#) to see if coded
- If coded - File as within normal range
- If not coded - Locate the provisional BCS: FOB result on [Consultations](#) > [Edit Consultation](#) > [Add Comment](#) > search for and add **Faecal Occult Blood: Negative** > **Save**. File as within normal range

Positive result

- If positive result check [Consultations](#) to see if coded
- If coded - File and type "**FOB Positive - follow up as per guidelines**"
- If not coded - Locate the provisional BCS: FOB result on [Consultations](#) > [Edit Consultation](#) > [Add Comment](#) > search for and add **Faecal Occult Blood: Positive** > **Save**. File and type "**FOB Positive - follow up as per guidelines**"

FOBT Non Response

- If non responder;
- If pt has a mobile number send an Accurx using '**Bowel Screening not done**' [template](#). File and type "**Accurx sent as third invite**" in comments
- If pt does not have a mobile number send a letter. [Consultations](#) > [Add Document](#) > [Create Letter](#) > search for '**FOBS Letter to Patient**' > Print or send via Docmail. **Save and close** letter to save to record. File and type "**Accurx sent as third invite**" in comments

INR (International Normalised Ratio) & Prothrombin time

INR

- Any INR regardless of result whether standalone or included on Prothrombin time forward to [Miss INR Results](#) in EMIS for the **Pharmacy team** to check

Cardio Lipins

Last Updated: 25.11.24

Cardio Lipins

- If normal file results
- If **abnormal**, send to the **Requestor**. If the requestor is off forward using the **Clinicians Protocol**

Infectious Mononucleosis

Infectious Mononucleosis

- If negative, file
- If **positive** forward using the **Clinicians Protocol**

Cervical Cytology Screen Test

Cytology

- Check **Consultations** to see which Nurse carried out the test.
- Forward to the nurse, by *Changing Owner*

Iron / transferrin

Iron / transferrin

- If normal file results
- If **abnormal**, send to the **Requestor**. If the requestor is off forward using the **Clinicians Protocol**

Immunoglobs

Immunoglobs

- If normal file results
- If **abnormal**, send to the **Requestor**. If the requestor is off forward using the **Clinicians Protocol**

DDimer

- If normal file results
- If **abnormal**, send to the **On Call Dr as this needs to be actioned urgently**. **NOTE:** Usually these are phoned through from the lab, so it may have already been actioned.

Coeliac / Ser tiss transglutaminase level

This is a specific test related to coeliac disease and may show as either of the two names above.

Coeliac

- **All results** regardless of result must be forwarded to the **Requestor**. If the requestor is off forward using the **Clinicians Protocol**

Quantiv Faecal Imm Tst (FIT Test)

FIT Test results

- If results are **below 10**, file as normal as "**within normal range (action as agreed with patient)**"
- If results are **above 10**, abnormal, send **urgent task** to reception to book an URGENT appointment with the requestor or alternative GP within 7 days. Use the text below on the task; File as 'Task Sent'
- **NOTE: the follow up appointment needs to be with a GP.** Where the requestor is not a GP use the relevant text below for the task.
 - **Task Text where requestor is a GP:**
 - Please book an urgent appointment - phone or F2F (Patient to choose) with [insert requestor name] or an alternative GP if on leave within 7days.
 - **Task Text where requestor is not a GP:**
 - Please book an urgent appointment - phone or F2F (Patient to choose) with a GP.

FIT Test non response

- Where the lab contact for a non-response from the patient, send the accurx "**FIT Test - Labs non response**"

Errors – Problem Set or Lab test req not done

Last Updated: 9.6.26

These indicate that the test has not been completed for some reason.

It needs to be established whether the test needs to be re booked or whether it needs to go back to Requestor.

NOTE It has been found that sometimes these error messages are sent incorrectly by the lab. Results show as not processed / completed but in fact the tests were completed and sent by the lab already. Or are being sent weeks or months after the bloods were taken and the test results have already been received. When an error is received, check the patients record to see if the test was received successfully by us in the previous day's / weeks. If so, the error message can be ignored.

Check
Consults

- Check Consultations to check the reason the test was taken;
- If for annual review - Send the patient an Accurx Self-book link for a F2F Blood Test **See Appointment Follow Up Protocol**. File as Accurx sent
- If a Clinician had requested the test - Send the patient an Accurx Self-book link for a F2F Blood Test **See Appointment Follow Up Protocol**. File as Accurx sent
- If test was Microscopy forward to the **Requestor** add sticky note - test not completed

Less frequent tests

Where abnormal results are shown for the below tests, forward using the **Clinicians Protocol**

Serum Prolactin

Se CEA

Serum para thyroid hormone

Specialised tests

Where a test appears that is not within the protocol, these will be specialised tests that a clinician will have ordered specifically for the patient. Always forward these to the **Requestor** unless they are on leave or the requestor is a Locum and then send to the Usual GP.

Examples are;

<i>Se Thyroid peroxidase Ab</i>	<i>IgG</i>
<i>IgA</i>	<i>Antibody level</i>
<i>Plasma Glucose</i>	<i>Rheumatoid Factor</i>
<i>H Pylori Antigen Test</i>	<i>Faecal Calprotectin Content</i>
<i>Coag Screen</i>	<i>Chlamydia Trachomatis NADA</i>
<i>Bone Profile</i>	<i>Antineutrophil Cytoplasmic Anti body level</i>
<i>AST Serum Level</i>	<i>Acetyl Chol Recep Ab</i>

Document control

Version	Date	Version created by	Comments
9 th June 26	9.6.26	Sharon Bean	Added T4 back into Thyroid monitoring
9 th June 26	9.6.26	Sharon Bean	Higher limit of 75 added to CRP
9 th June 26	9.6.26	Sharon Bean	Note to check for completed tests when errors are received from lab
6 th July 26	6.7.26	Sharon Bean	Removed reference to named locums for sending labs
18 th August 26	18.8.26	Sharon Bean	Hba1c updated first box to 47 for pre DM